

Policy on Patient and Family Rights and Responsibilities

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Patient Rights Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» documents and displays patient and family rights and responsibilities, makes staff aware of their duty to protect these rights, actively promotes them, and provides a fair mechanism to report and act on any violation.

1. Purpose

This policy explains how «Hospital Name» protects and promotes patient and family rights and informs them about their responsibilities during care: documenting and displaying rights and responsibilities, actively promoting them through counselling and accessible material, protecting rights through staff conduct, providing a mechanism to report violations, and ensuring top leadership monitors and acts on violations.

Informed consent, privacy during examinations, and other decision-making rights are covered in the hospital's other policies.

2. Scope

This policy applies to all staff who interact with patients and families, the Patient Rights Officer, ward and OPD staff responsible for display and counselling, and top leadership responsible for monitoring rights violations.

3. Policy standards

«Hospital Name» documents, displays, and actively promotes patient and family rights and responsibilities in a format and language patients and families can understand, and protects these rights through staff awareness and conduct.

A mechanism exists for patients and families to report a violation of their rights, and top leadership monitors, investigates, and takes corrective and preventive action on every reported violation.

4. Non-negotiable rules

- Do not display patient rights and responsibilities in a single language only — information, education and communication material shall be at least bilingual.
- Do not limit rights promotion to a static display board — in-patients shall receive counselling and out-patients shall have accessible educational material.
- Do not leave staff untrained on their duty to protect and respect patient and family rights.
- Do not operate without a mechanism for patients and families to report a rights violation.
- Do not leave a reported violation uninvestigated, undocumented, or without corrective/preventive action from top leadership.

5. What we do

5.1 Document and display patient and family rights and responsibilities

Patient and family rights and responsibilities are documented in consonance with the Charter of Patients' Rights (National Human Rights Commission, 2018). They are displayed prominently where visible to patients, families and visitors. Information, education and communication material is at least bilingual.

5.2 Actively promote patient and family rights and responsibilities

Beyond static display, «Hospital Name» takes active steps to promote rights and responsibilities. In-patients and/or their family are counselled on their rights and responsibilities in a format and language they can understand. For out-patients, educational material is easily accessible and prominently displayed in OPD.

5.3 Protect patient and family rights through staff conduct

Staff are made aware of their responsibility in protecting and respecting patient and family rights. Staff conduct themselves in a manner that conveys a positive attitude toward the protection of these rights.

5.4 Provide a mechanism to report rights violations

A mechanism exists for patients and/or family to report a violation of their rights. Violations are reported through an incident reporting form providing details of how the right was violated and, where applicable, by whom.

5.5 Monitor, investigate and act on rights violations

Top leadership documents violations of patient and family rights, investigates them, and maintains records of each incident and its outcome. Corrective and/or preventive action is taken in a time frame defined by «Hospital Name».

6. Stop-work authority

Not applicable. This standard has no stop-work authority — violations of patient and family rights are addressed through the reporting mechanism (Section 5.4) and leadership review (Section 5.5), not through an immediate stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring patient and family rights are documented, displayed, promoted, and that reported violations receive leadership attention.
Patient Rights Officer	Owns day-to-day implementation of this policy; maintains the rights charter, display boards, and counselling records; receives violation reports.
Top Leadership / Governing Body	Reviews the violation log; investigates and closes CAPA arising from reported violations.
Nursing Superintendent	Ensures ward staff are trained on patient rights and complete counselling for in-patients.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Display and documentation	Rights and responsibilities documented per the Charter of Patients' Rights; display boards current and bilingual.
Active promotion	In-patient counselling records; OPD educational material accessibility.
Staff awareness	Staff training records on rights protection.
Reporting mechanism	Violation log entries; accessibility of the

	reporting mechanism to patients/families.
Leadership review	Leadership review minutes; CAPA records with owners and closure evidence.

9. Training and staff acknowledgement

All staff shall be familiar with patient and family rights and responsibilities, their duty to protect these rights, and the mechanism to report a violation, at induction and periodically.

I have read the Policy on Patient and Family Rights and Responsibilities of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical and non-clinical staff, the Patient Rights Officer, ward and OPD in-charges, and top leadership.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
IEC	Information, Education and Communication
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Out-Patient Department

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.1.a	Commitment*	Sections 3 and 5.1 address documentation, display, and bilingual IEC material for patient and family rights and responsibilities.
PRE.1.b	Achievement*	Sections 3 and 5.2 address active promotion through in-patient counselling and accessible OPD material.

PRE.1.c	CORE	Sections 3 and 5.3 address staff awareness and conduct protecting patient and family rights.
PRE.1.d	CORE	Sections 3 and 5.4 address the mechanism for patients and families to report a rights violation.
PRE.1.e	CORE	Sections 3 and 5.5 address leadership documentation, investigation, and CAPA for reported violations.

13. Required Records/Evidence Checklist

Documentation and display — PRE.1.a (Commitment*)

- Documented set of patient and family rights and responsibilities, in consonance with the Charter of Patients' Rights.
- Display boards visible to patients, families and visitors, in at least two languages.
- Awareness records with date, language and named staff.

Active promotion — PRE.1.b (Achievement*)

- In-patient counselling records (format, language, named staff).
- OPD display records confirming accessible educational material.

Staff awareness and conduct — PRE.1.c (CORE)

- Staff training records on protecting and respecting patient and family rights.
- Conduct or incident investigation records where relevant.

Reporting mechanism — PRE.1.d (CORE)

- Written mechanism for patients/families to report a violation (complaint box, helpline, form).
- Violation log with entries showing how the right was violated and, where applicable, by whom.

Leadership monitoring and CAPA — PRE.1.e (CORE)

- Dated leadership review minutes.
- CAPA records with owners and closure evidence.
- Violation log referenced during leadership review.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.1.

2. Charter of Patients' Rights, National Human Rights Commission (2018), circulated by the Ministry of Health and Family Welfare to States/UTs for adoption (2019).

Disclaimer

This policy reorganises the supplied PRE.1 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

This policy shall be reviewed at least once every year, or earlier if NABH standards, the Charter of Patients' Rights, or applicable law changes.

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